

Current Yellow fever situation in the Americas: call for strengthened surveillance and vaccination efforts in enzootic and newly affected areas

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Yellow fever activity in the Americas Region has intensified over the past two years, with 2025 recording the highest annual number of confirmed cases since 2020 (Figure 1). This resurgence follows major outbreaks reported in 2017 and 2018 and is notable for cases in areas without a historical record of transmission, including outside of the Amazon Region. Preventing further spread of the virus requires maintaining strong surveillance systems, sustaining high vaccination coverage in populations living in high-risk areas, and ensuring a timely detection and coordinated response.

Between epidemiological week (EW) 1 and EW 53 of 2025, a total of 346 confirmed human cases of yellow fever and 143 deaths (case fatality rate [CFR] 41%) were reported across seven countries of the Americas Region. This represents a 5.6-fold increase compared with 2024, when 61 confirmed cases were reported. Countries reporting cases in 2025 included Bolivia, Bolivarian Republic of Venezuela, Brazil, Colombia, Ecuador, Guyana, and Peru. (Figure 2).

In the first seven weeks of 2026, 34 confirmed human cases including 15 deaths (CFR 44%) were reported in four countries: Bolivia, Colombia, Peru, and Venezuela. The continued occurrence of cases in the first weeks of 2026 indicates ongoing transmission and highlights the need for sustained surveillance across the Region (Figure 3).

Beyond the overall increase in cases, recent epidemiological patterns reveal the occurrence of yellow fever cases into areas not historically considered at risk. Since 2024, cases have been observed in a wider geographical area than in previous years, including high-elevation forest areas associated with the Bosque de Galilea

region in Tolima, Colombia. In Brazil, most cases reported in 2025 occurred across 29 municipalities in São Paulo State; in Venezuela, cases have also expanded geographically, spanning multiple states and areas previously not classified as risk zones.

In parallel, epizootic events in nonhuman primates have been reported across several countries, particularly in Bolivia, Bolivarian Republic of Venezuela, Brazil, and Colombia. In multiple settings, primate mortality preceded or coincided with human cases.

In ecological perspective, these events across an extended area of the continent highlight ongoing sylvatic transmission and spillover at forest edges, where human activities intersect with biodiverse ecosystems. These environments provide ideal conditions for canopy-dwelling mosquitoes to transmit the virus from nonhuman reservoirs to human populations.

DISCUSSION

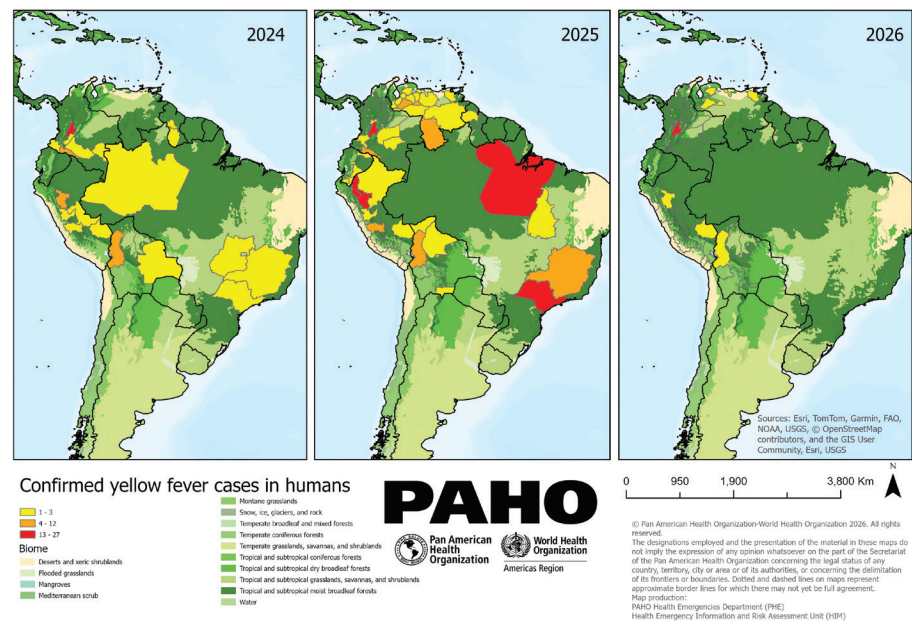
Although the number of cases reported in 2025 remains comparable to previous years with elevated incidence — such as 1966, 1998, and 2003 — the current epidemiological situation points to continued transmission and potential geographic expansion of yellow fever in the Americas Region. At the same time, since mid-2024 the pattern has been characterized by expansion into new ecological areas and persistent outbreaks in localized hotspots.

One of the most significant outbreaks is occurring in Tolima, Colombia, where sustained transmission has been documented since 2024. Between 2024 and early 2026, more than 150 confirmed cases, including a substantial number of deaths,

¹ PAHO Health Emergencies Department (PHE), through the Health Emergency Information & Risk Assessment Unit (HIM) and Infection Hazard Management Unit (IHM), and Comprehensive Immunization Special Program.

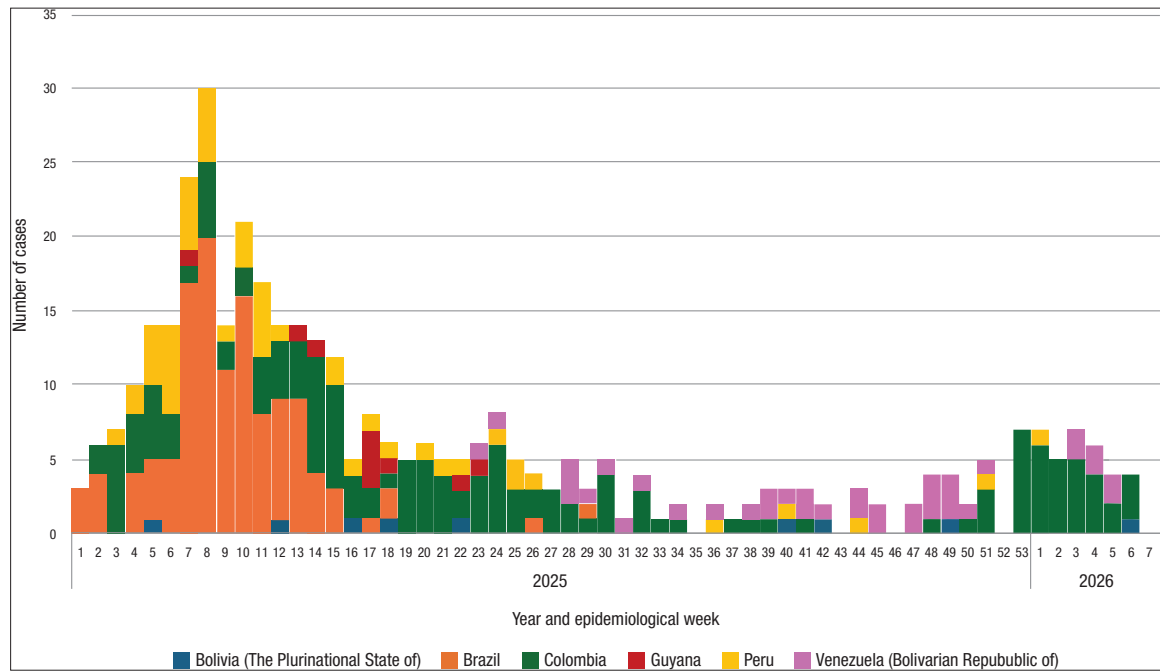
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FIGURE 1. Confirmed cases of yellow fever in humans by year in the Americas Region, between 2024 and 2026 (as of EW 7 of 2026).



Source: Adapted from data provided by the respective countries and reproduced by PAHO/WHO.

FIGURE 2. Yellow fever cases by year, country, and EW of symptom onset, in the Americas Region, 2025–2026.



*Note: Includes only cases for which information on symptom onset by epidemiological week is available.
Source: Adapted from data provided by the respective countries and reproduced by PAHO/WHO.

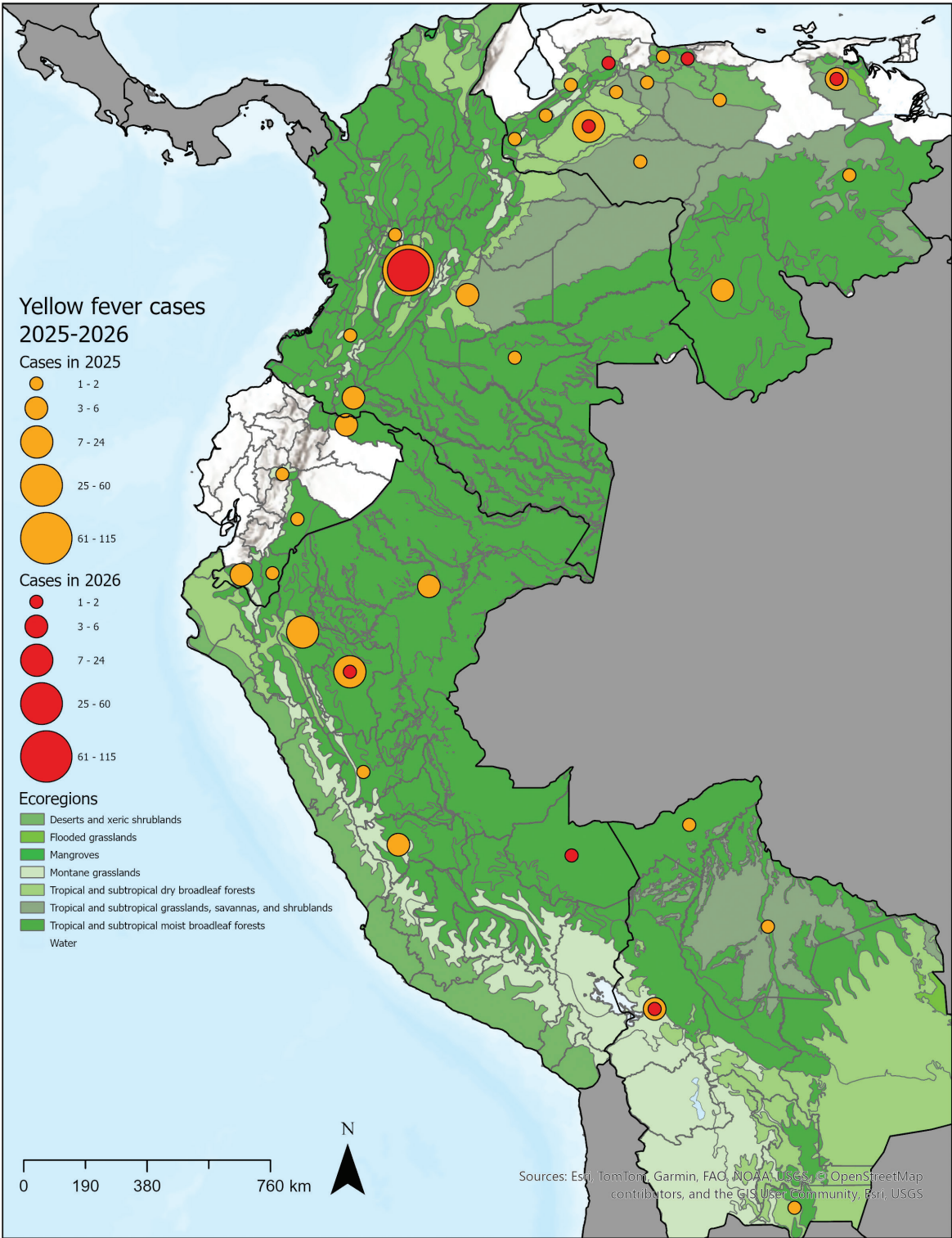
have been reported. This outbreak has affected multiple municipalities and remains associated with forested environments bordering protected ecological areas.

Another important epidemiological signal is the occurrence of epizootics in non-human primates, which serve as early indicators of viral circulation. This reinforces the importance

of integrated surveillance and a One Health approach to guide timely, targeted vaccination and public health response.

Several contextual factors may facilitate further spread of yellow fever in the Region. These include environmental conditions favorable for vector proliferation, expansion of human activities into forested areas (such as agriculture, mining, and logging), increased

FIGURE 3. Confirmed cases of yellow fever in humans in Bolivia, Colombia, Ecuador, Peru, and Venezuela, 2025-2026 (as of EW 7 of 2026).



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The designations employed and the presentation of the material in these maps do not imply the expression of any opinion whatsoever on the part of the Secretariat of the Pan American Health Organization concerning the legal status of any country, territory, city or area or of its authorities, or concerning the delimitation of its frontiers or boundaries. Dotted and dashed lines on maps represent approximate border lines for which there may not yet be full agreement.
Map production:
PAHO Health Emergencies Department (PHE)
Health Emergency Information and Risk Assessment Unit (HIM)

Source: Adapted from data provided by the respective countries and reproduced by PAHO/WHO.

Key takeaways:

- Increasing regional activity: Yellow fever cases increased significantly in 2025 compared with 2024, with continued transmission reported in early 2026.
- Geographic expansion of transmission: Cases are being detected in areas outside traditional endemic zones, including high-altitude forest and peri-urban regions.
- Persistent sylvatic transmission: Human infections are primarily associated with spillover events at forest edges where human activity intersects with sylvatic transmission cycles.
- Epizootics remain a key early warning signal: Surveillance of non-human primates continues to provide critical information for anticipating human outbreaks.
- Vaccination, surveillance, and risk communication remain central to prevention: Maintaining high vaccination coverage, strengthening surveillance systems, and effectively engaging communities, are essential to prevent further spread and reduce mortality.

population mobility, and gaps in vaccination coverage in populations living or working in high-risk environments. In addition, the detection of cases in areas close to urban centers raises concerns about the potential for urban transmission cycles, particularly in settings where *Aedes aegypti* populations remain widespread.

In response to the evolving epidemiological situation, PAHO issued an Epidemiological Alert urging Member States to strengthen surveillance, intensify vaccination efforts in at-risk populations, and ensure preparedness of health services to detect and manage cases of yellow fever.

RECOMMENDATIONS

Given the current situation, Member States are encouraged to reinforce surveillance and response capacities across several key areas. This includes maintaining epidemiological surveillance systems capable of detecting suspected cases rapidly and conducting timely investigations. Strengthening laboratory diagnostic capacity is also vital, particularly the use of molecular detection methods during the early viremic phase of the disease. Also, ensuring tissue samples from fatal cases is critical to confirm the infection or to address differential diagnosis.

In areas with limited laboratory capacity, decentralization of testing to subnational levels may facilitate faster confirmation of cases and improved outbreak response. Because early detection of viral circulation in wildlife can trigger preventive vaccination campaigns before human cases occur, implementing and strengthening surveillance of epizootics in non-human primates remains a key priority.

Vaccination remains the most effective public health intervention for preventing yellow fever. Countries should ensure high vaccination coverage in populations living in endemic and high-risk areas and maintain strategic vaccine stockpiles to support routine immunization and rapid response during outbreaks. Case fatality remains high, reinforcing the need to strengthen early clinical recognition and ensure close clinical

and laboratory monitoring, with careful attention to warning signs. Strengthening access to blood bank support and timely management of complications are essential to reduce mortality among patients with severe disease.

Risk communication and community engagement are critical components of yellow fever prevention. Informing populations about vaccination recommendations and preventive measures, particularly for workers exposed to forest environments, can help reduce the risk of infection.

Finally, continued regional collaboration and information sharing under the International Health Regulations (2005) remain essential to monitor cross-border risks and coordinate response measures across the Americas.

Data Availability Statement. Data available from Epidemiological Alert on Yellow Fever in the Americas

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